

PARTIAL SUSPENSION

HRT and Menopause · Tostran arm only

The Tostran (testosterone) arm of this PGD must not be used until further notice.

The Evorel Conti, Evorel Sequi, Evorel and Utrogestan arms are NOT affected and remain in use.

Reason

Testosterone is a Schedule 4 Part II controlled drug. Whether a Patient Group Direction may lawfully supply it is disputed and is being confirmed. NHS Specialist Pharmacy Service guidance updated 1 July 2026 lists, for Schedule 4, ·all drugs except anabolic steroids· as permitted under a PGD. Schedule 4 Part II is the anabolic steroids part.

This replaces the correction notice issued earlier on 7 September 2026

That notice restricted the document to pharmacists and stated that every other provision remained in force. That assurance was not verified and should not have been given.

What to do now

Do not supply Tostran under this PGD. Refer women requiring testosterone for HSDD to a prescriber. Continue to use the four HRT arms normally.

Two further defects in this document, being corrected separately

The Tostran and Utrogestan arms both state a maximum treatment period of ·3 days·, which is template text from an acute condition. For Utrogestan this is serious: three days **of progestogen gives no endometrial protection. Supply the stated 30-capsule cycle and disregard the 3-day figure.**

There is no rule anywhere in this document requiring progestogen opposition for a woman **with an intact uterus. Do not supply oestrogen-only Evorel to a woman who has a uterus.**

Issued 7 September 2026 · Get Real Health

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Evorel Conti and Evorel plus Utrogestan for the treatment of Menopause, the administration of Evorel Sequi for Perimenopause and the administration of testosterone

Summary of NICE / NICE CKS Guidelines for Menopause

Overview

- **Menopause** is defined as **12 months of amenorrhoea** due to permanent loss of ovarian function.
- Average age: **51 years**.
- **Perimenopause** refers to the transition phase with **menstrual irregularity** and **vasomotor symptoms**.

Symptoms

- **Vasomotor**: hot flushes, night sweats
- **Menstrual changes**
- **Psychological**: low mood, anxiety, irritability, poor concentration
- **Genitourinary**: vaginal dryness, dyspareunia, urinary frequency
- **Musculoskeletal**: joint aches
- **Sleep disturbance**
- Reduced libido

Diagnosis

- **Clinical diagnosis** in women over **45** with typical symptoms — **no need for blood tests**.
- Consider **FSH testing** only if:
 - Aged 40–45 with menopausal symptoms
 - <40 and suspected **premature ovarian insufficiency (POI)**

Management Approach

1. Lifestyle and Supportive Measures

- Healthy diet and exercise
- Smoking/alcohol reduction
- CBT for mood/sleep disturbance
- Vaginal moisturisers/lubricants for local symptoms

2. Hormone Replacement Therapy (HRT)

Indications:

- First-line treatment for **vasomotor symptoms** and other menopausal symptoms affecting quality of life
- Prevention/treatment of **osteoporosis** in high-risk women when other options unsuitable

Types of HRT:

Woman's Status	Regimen
Has uterus	Combined oestrogen + progestogen to protect endometrium
No uterus	Oestrogen-only HRT
Perimenopausal	Cyclical HRT
Postmenopausal	Continuous combined HRT (after 12 months of amenorrhoea)

Routes:

- **Oral or transdermal** (patch/gel/spray)
- **Transdermal** preferred if:
 - Risk factors for VTE
 - High triglycerides
 - Liver disease
 - Patient preference

Local oestrogen:

- For **vaginal symptoms** (e.g. dryness, atrophy)
- Can be used **alone or with systemic HRT**
- Safe for long-term use with minimal systemic absorption

Risks and Benefits of HRT

- Benefits:

- Relief of symptoms
 - Bone protection
- Risks:
 - Slight increased risk of **VTE, stroke, breast cancer** (with long-term combined HRT)
 - Risk profile varies by **type, dose, route, and duration**

HRT is **safe and effective for most women under 60** when initiated appropriately.

Alternatives to HRT

- For women who **can't or choose not to use HRT**:
 - **SSRIs/SNRIs** (e.g. venlafaxine, fluoxetine)
 - **Clonidine** (limited evidence)
 - **Gabapentin** (for vasomotor symptoms)
 - CBT for psychological symptoms
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Premature Ovarian Insufficiency (POI)

- Defined as menopause **before age 40**
 - Offer **HRT or COCP** until at least the **average age of natural menopause** (around 51)
 - Refer to specialist if needed
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Follow-Up and Monitoring

- Review at **3 months**, then **annually**
 - Monitor symptom control, side effects, and ongoing need
 - Reassess risks and benefits periodically
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When to Refer

- Uncontrolled symptoms despite treatment
- Complex history or contraindications to HRT
- Suspected POI or menopause under 40
- Osteoporosis management in context of menopause

Patient Group Direction

for the supply/administration of



Evorel Conti (estradiol/norethisterone transdermal patch)

for the treatment of

Menopause

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Evorel Conti is indicated for hormone replacement therapy (HRT) in postmenopausal women for the relief of oestrogen deficiency symptoms. It is also used in the prevention of osteoporosis in high-risk women who are intolerant of or contraindicated for other therapies.
Inclusion criteria	- Women aged 50 years and over who are postmenopausal. - Presence of moderate to severe menopausal symptoms impacting quality of life. - Informed consent obtained. - No contraindications to HRT.
Exclusion criteria	- Known, suspected, or history of breast cancer or oestrogen-dependent tumours. - Active or recent arterial thromboembolism or venous thromboembolism. - Liver dysfunction or disease. - Undiagnosed vaginal bleeding. - Known hypersensitivity to estradiol, norethisterone, or patch adhesive.
Cautions	- Use the lowest effective dose for the shortest duration. - Monitor blood pressure and review symptom control and side effects regularly. - Discuss risks and benefits including breast cancer and cardiovascular risk. - Educate on proper patch application and disposal.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Evorel Conti
Legal category	POM
Storage	Do not store above 25°C. Store in original packaging to protect from light.
Route/method of administration	Transdermal patch applied to clean, dry, hairless area of the lower abdomen or buttocks.
Dose and frequency	Apply one patch twice weekly (change every 3 to 4 days). Continuous combined HRT regimen.
Quantity to be administered and/or supplied	Eight patches for 4 weeks (monthly supply).
Maximum or minimum treatment period	Assess benefit/risk at 3–6 months, then annually during long-term use.

Adverse effects	Breast tenderness, headache, nausea, mood changes, skin irritation, breakthrough bleeding. Rare: thromboembolic events, gallbladder disease. Refer to SmPC.
Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8//2025

Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025
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Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of



Evorel Sequi (estradiol and estradiol/norethisterone
transdermal patches)

for the treatment of

Perimenopause

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Evorel Sequi is indicated for hormone replacement therapy (HRT) for oestrogen deficiency symptoms in postmenopausal women with an intact uterus, particularly during the early years of menopause. It is also used to prevent osteoporosis in high-risk women intolerant of or contraindicated for other therapies.
Inclusion criteria	- Women aged 50 years and over who are postmenopausal and have an intact uterus. - Presence of moderate to severe menopausal symptoms. - Informed consent obtained. - No contraindications to HRT.
Exclusion criteria	- Known or suspected breast cancer or oestrogen-dependent tumour. - Active or recent thromboembolic disease. - Liver impairment or disease. - Undiagnosed vaginal bleeding. - Hypersensitivity to estradiol, norethisterone, or adhesives.
Cautions	- Monitor blood pressure, bleeding pattern, and menopausal symptoms regularly. - Use lowest effective dose for the shortest duration. - Inform about possible risks including thromboembolism and hormone-sensitive cancers. - Ensure proper patch application, rotation, and disposal.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Evorel Sequi
Legal category	POM
Storage	Do not store above 25°C. Store in original packaging to protect from light.
Route/method of administration	Transdermal patch applied to clean, dry, hairless area of the lower abdomen or buttocks.
Dose and frequency	Apply one Evorel patch twice weekly for the first 2 weeks, then one Evorel Conti patch twice weekly for the next 2 weeks. Repeat monthly (sequential HRT regimen).
Quantity to be administered and/or supplied	Eight patches (four Evorel and four Evorel Conti) for 4 weeks (monthly supply).
Maximum or minimum treatment period	Assess benefit/risk at 3–6 months, then annually during long-term use.

Adverse effects	Breast tenderness, headache, mood changes, nausea, skin irritation, irregular bleeding. Rare: thromboembolic events, gallbladder disease. Refer to SmPC.
Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

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Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/6/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Evorel (estradiol-only transdermal patch)

for the treatment of

Menopause

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Evorel is indicated for hormone replacement therapy (HRT) in postmenopausal women with a history of hysterectomy, for the relief of oestrogen deficiency symptoms and for the prevention of osteoporosis in high-risk women intolerant of other treatments.
Inclusion criteria	- Women aged 50 years and over who are postmenopausal and have had a hysterectomy. - Experiencing moderate to severe menopausal symptoms. - Informed consent obtained. - No contraindications to oestrogen-only HRT.
Exclusion criteria	- Known, suspected, or history of breast cancer or oestrogen-dependent tumour. - Active or recent arterial or venous thromboembolic disorder. - Severe liver disease. - Known hypersensitivity to estradiol or patch adhesive.
Cautions	- Monitor symptom control and side effects regularly. - Use lowest effective dose for shortest appropriate duration. - Educate on patch rotation, application, and disposal. - Consider calcium/vitamin D intake and bone health monitoring in osteoporosis prevention.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Evorel
Legal category	POM
Storage	Do not store above 25°C. Store in original packaging to protect from light.
Route/method of administration	Transdermal patch applied to clean, dry, hairless area of the lower abdomen or buttocks.
Dose and frequency	Apply one patch twice weekly (change every 3 to 4 days). Continuous oestrogen-only HRT regimen.
Quantity to be administered and/or supplied	Eight patches for 4 weeks (monthly supply).
Maximum or minimum treatment period	Assess benefit/risk at 3–6 months, then annually during long-term use.
Adverse effects	Breast tenderness, headache, nausea, mood changes, skin irritation, vaginal discharge. Rare: thromboembolism, endometrial hyperplasia. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Tostran gel

for the treatment of

Hypoactive sexual desire disorder (HSDD) in postmenopausal
women

Healthcare professionals covered and training

Patient Group Direction for the administration of Tostran	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Tostran is indicated for the treatment of hypoactive sexual desire disorder (HSDD) in postmenopausal women with low serum testosterone levels and reduced sexual desire, where other causes have been excluded and benefits outweigh risks. Diagnosis should be confirmed by a specialist or according to local protocols.
Inclusion criteria	- Postmenopausal women aged 45 years or older. - Diagnosis of HSDD confirmed and documented. - Informed consent obtained after discussing off-label use. - Baseline serum testosterone measured. - No contraindications to testosterone therapy.
Exclusion criteria	- History or presence of breast or endometrial cancer. - Active liver disease or significant hepatic impairment. - Known or suspected androgen-sensitive neoplasia. - Uncontrolled cardiovascular disease. - Known hypersensitivity to testosterone or any component of the gel. - Current use of oral or transdermal estrogen without clinical review.
Cautions	- Monitor serum testosterone at baseline, 6–12 weeks after starting, and every 6–12 months thereafter. - Monitor for signs of virilisation (e.g., acne, hirsutism, voice deepening). - Apply to intact skin and avoid contact with partners. - Stop treatment if testosterone levels exceed physiological female range or if adverse effects occur.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Tostran gel
Legal category	POM
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Topical – apply to clean, dry, intact skin on the lower abdomen or upper thighs
Dose and frequency	Apply 0.5 g of gel (10 mg testosterone) every other day
Quantity to be administered and/or supplied	Dispense 60 g metered-dose canister
Maximum or minimum treatment period	3 days; reassess if no improvement or if symptoms recur
Adverse effects	Nausea, headache, flatulence, diarrhoea, and Acne, hirsutism, voice deepening, mood changes, application site reactions. Rare:

	hepatotoxicity, virilisation.
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Utrogestan (Utrogestan 100 mg modified-release capsules)

for the treatment of

Menopause

Healthcare professionals covered and training

Patient Group Direction for the administration of Utrogestan	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training with Get Real Health in order to use this PGD. This training will be documented and signed off by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Utrogestan is indicated for the prevention of endometrial hyperplasia in postmenopausal women with an intact uterus who are receiving continuous estrogen-only hormone replacement therapy (HRT).
Inclusion criteria	- Postmenopausal women aged 45 years or older. - Intact uterus. - Currently prescribed continuous estrogen-only HRT. - Able to give informed consent. - No contraindications to progesterone therapy.
Exclusion criteria	- History or presence of breast or endometrial cancer. - Unexplained vaginal bleeding. - Severe hepatic impairment or active liver disease. - History of thromboembolic disorders. - Known hypersensitivity to progesterone or any capsule ingredients.
Cautions	- Use with caution in patients with a history of depression. - Monitor for common side effects such as drowsiness, bloating, or breast tenderness. - Advise taking capsule at bedtime on an empty stomach. - Review response and adherence at least annually.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Utrogestan 100 mg modified-release capsules
Legal category	POM
Storage	Store below 30°C in original packaging to protect from light and moisture.
Route/method of administration	Oral – swallow capsule whole at bedtime on an empty stomach
Dose and frequency	100 mg at bedtime once daily (continuous regimen)
Quantity to be administered and/or supplied	30 capsules per 30-day cycle
Maximum or minimum treatment period	3 days; reassess if no improvement or if symptoms recur
Adverse effects	Nausea, headache, flatulence, diarrhoea, and Drowsiness, breast tenderness, bloating, headache. Rare: mood changes, dizziness, hypersensitivity.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk
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Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates

Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025